



Week 1:

Communicating with Patients as
Medical Providers

Sign into iClicker: NPDW
Please put away your devices 😊



WRITING PCRS

PCRs

When?- For any patient for whom you perform a medical assessment or intervention beyond simple First Aid (i.e. anything beyond handing out a band-aid/Ice Pack)

When not to?- Simple First Aid/comfort items requested by patients (i.e band-aids, Ice Packs) who refuse and/or do not require assessment



Key Takeaways/Common Mistakes

- Demographics information required for every patient. Try your best to acquire.
 - Pro Tip: Ask for their ID!
- Chief complaint should be in quotes (“I drank too much,” not EtOH). If unresponsive/incomprehensible, document as: “None (Unresponsive/Incomprehensible)”

PATIENT CARE REPORT		BERKELEY MEDICAL RESERVE CORPS			
NAME		SEX	AGE	TIME OF CALL	
PHONE		DOB		LOCATION	
STREET		APT / STE #		CALLER	
CITY		ZIP		INTAKE BY	
CHIEF COMPLAINT					
FOUND / ARRIVED: ☐ ALONE ☐ COMPANION(S) ☐ STAFF ☐ SECURITY ☐ BMRC					
MODE: ☐ AMBULATORY ☐ ASSISTED ☐ WHEELCHAIR ☐ GURNEY ☐ C-SPINE					
CONDITION: ☐ ALERT ☐ AGITATED ☐ COMBATIVE ☐ LETHARGIC ☐ AMS ☐ UNCONSCIOUS					



Key Takeaways/Common Mistakes

- If A+O status less than 4. Document exactly what the patient could not report/what led you to conclude the patient was altered. (E.g: “Unaware of current location/city”).
- In systems assessment details and findings, document signs and symptoms **AND** pertinent negatives. (i.e Cardiovascular: “Pedal edema.” Neurological: “CSMs in all extremities intact”).

WNL	ABN	NA	DETAILS / FINDINGS		WNL	ABN	NA	DETAILS / FINDINGS	
1. NEUROLOGICAL			A / O x	V P U	GCS:				
☐	☐	☐				☐	☐	☐	
2. CARDIOVASCULAR					CP ☐				
☐	☐	☐				☐	☐	☐	
3. RESPIRATORY			☐ EVEN, UNLABORED ☐ SHALLOW	☐ LABORED ☐ RAPID	☐ ASSISTING ☐ RETRACTING				
☐	☐	☐	BREATH SOUNDS:		SOB ☐	6. SKIN			
						☐	☐	☐	



Key Takeaways/Common Mistakes

- **Difference between “WNL” and “NA”:** Tick WNL (Within Normal Limits) if it’s a system that is relevant and you have assessed is normal (i.e presence of respiratory distress for a chest pain patient). Tick NA (Not Applicable/Assessed) if it’s completely irrelevant (i.e extremities AKA deformities for a purely medical call).
- **Allergies, medications, past medical history:** If none, don’t leave blank, write: “None reported/Pt denies”

WNL	ABN	NA	DETAILS / FINDINGS		WNL	ABN	NA	DETAILS / FINDINGS	
1. NEUROLOGICAL			A / O x	V P U	GCS:		4. GI / GU		
☐	☐	☐			☐	☐	☐		
2. CARDIOVASCULAR					CP ☐	5. EXTREMITIES			
☐	☐	☐			☐	☐	☐		
3. RESPIRATORY			☐ EVEN, UNLABORED ☐ SHALLOW	☐ LABORED ☐ RAPID	☐ ASSISTING ☐ RETRACTING	6. SKIN			
☐	☐	☐	BREATH SOUNDS:		SOB ☐	☐	☐	☐	
ALLERGIES:					PAST MEDICAL HISTORY				
MEDICATIONS:					☐ ASTHMA / COPD	☐ CVA (STROKE)	☐ GI / GU	☐ SEIZURE	
					☐ CARDIAC (MI ☐)	☐ DIABETES	☐ HYPERTENSION	☐ PSYCHIATRIC	



NARRATIVE STRUCTURE (all PCR's should follow this general narrative format):

- **NARRATIVE STRUCTURE** (all PCR's should follow this general narrative format):
 1. **MRC [1] [dispatched/flagged down]** to/for [“X YOM/F”] @ [Location]
 2. **AOS (Arrived on Scene) to find** [X YOM/F].... initial impressions (How they're positioned, level of consciousness/A.O. status, ABCs, skin signs etc.)... w/ C.C. (Chief Complaint): [“_____”]
 3. **Upon assessment,** -> insert all relevant signs and symptoms, answers to assessment questions and pertinent negatives here, especially if not documented/elaborated upon elsewhere. Less relevant items that are documented in Vitals or Details & Findings may be skipped for redundancy

TIME	HR	BP	RR	SPO2	°C / °F	NARRATIVE: IMPRESSIONS / ASSESSMENTS / TREATMENTS / RESPONSES



NARRATIVE STRUCTURE (continued)

4. (If applicable) **ALS requested** via [UCPD emergency line/911/radio] @ [time you requested ALS.]. Put this right after AOS/initial impressions if you're requesting ALS immediately upon arrival.

5. Transferred care to [BER (Berkeley Fire) Engine/Medic #] or “Pt released after treatment and refused additional service” If releasing pt, document circumstances that allow the pt to be safely released without further ALS assessment/intervention.

6. Notes: If moving pt, document as: “Pt securely loaded onto [gurney/stair chair] and moved to [other location] without incident”

						☐ CALL FOR ALS / TRANSPORT
DISCHARGE DETAILS		DESTINATION:		☐ ALONE ☐ COMPANION(S) ☐ POLICE (BADGE:) ☐ STAFF		
TIME:	CONDITION:	☐ STABLE	☐ UNSTABLE	☐ EMS AGENCY: _____ RIG #: _____		
I authorize treatment by Berkeley Medical Reserve Corps staff to perform any medical intervention/treatment which is necessary in their professional judgement. ☐ YES		I acknowledge that any medical information taken by the Berkeley Medical Reserve Corps will be held confidential and may be shared with third parties for the purpose of providing medical care. ☐ YES		SIGNATURE: _____ DATE: _____		

NARRATIVE STRUCTURE (continued)

- **Pt Signatures:** MUST be obtained or document why unable to obtain
 - **When transferring care:** Document who you transfer care to and destination if possible
 - **Pt Unable to Sign:** If pt is minor/ALOC/AMS, they do not have the legal/mental capacity to sign. Document as “Pt unable to sign (reason).” Legal guardians or chaperones empowered to make medical decisions on a minor’s behalf in parents’ absence, may sign.
- **Provider Signatures:** All providers involved in care should sign and approve PCR
- **ACRONYMS:** Adhere to [ACPHD Approved Acronyms List](#)



BMRC STAFF NAMES & SIGNATURES, AND NAMES & LICENSE #s / PHONE #s OF ANY ADDITIONAL SUPPORTING HEALTHCARE PROVIDERS:

NARRATIVE STRUCTURE (continued)

- **AMA:** patients refuse treatment
 - Must be A.O.X4

REFUSAL OF TREATMENT

_____ I have been advised that medical care on my behalf is necessary, and that refusal of care and assistance could be hazardous to my health, and under certain circumstances, include disability or death. **OR** I acknowledge that I may have a medical problem which may require additional medical attention, and that an ambulance could be arranged to take me to the hospital. Instead, I elect to seek alternative medical care and/or refuse evaluation/treatment and/or transport.

_____ By signing this form, I am releasing the Berkeley Medical Reserve Corps of any liability or medical claims resulting from my decision to refuse care against medical advice.

SIGNATURE

PRINTED NAME

DATE



Scope of practice



Giving patients random medicines

CPR

Giving patients *their own* medicine

Using AEDs

Telling people whether or
not to call 911

Giving oxygen

Calling 911

Applying tourniquets

Applying bandages

Using a BVM

Applying a splint

Using an Epi-Pen

Giving naloxone (aka narcan)

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CPR

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Applying * a splint

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Using an Epi-Pen

*
Giving naloxone (aka narcan)

Membership Guidelines

Director of Administration



General Member Requirements

- Attend 6 or more weekly training sessions (6 weeks of trainings, attending both Monday and Tuesday does not count.)
 - Do not need to be in the DeCal.
- Working 3 MedOps events or equivalent
 - 1 ride along (with reflection), 1 work shift (EMS related), 4 hours of Public Health events (or 2 events x .5 events each), and/or 1 Rock Medicine shift may be used to substitute up to TWO MedOps events
- Attending MCI drill or submitting alternate assignment



New Member Requirements

- Attend all weekly training sessions AND be enrolled in the DeCal
 - 2 unexcused absences allowed
- Submit all assignments on bCourses
- Working 3 MedOps events or equivalent
 - 1 ride along (with reflection), 1 work shift (EMS related), 4 hours of Public Health events (or 2 events x .5 events each), and/or 1 Rock Medicine shift may be used to substitute up to TWO MedOps events
- Attending MCI drill or submitting alternate assignment



What counts as an event related violation?

These expectations apply across general members, Public Health instructors, PHO/FTO's, and CPR instructor responsibilities as applicable

- Arriving after call time without warning
- Arriving after the class/event has started
- No-show
- Same-day cancellation (unless there is a valid emergency)
- Minor unprofessional conduct
- Violating responsibilities of role

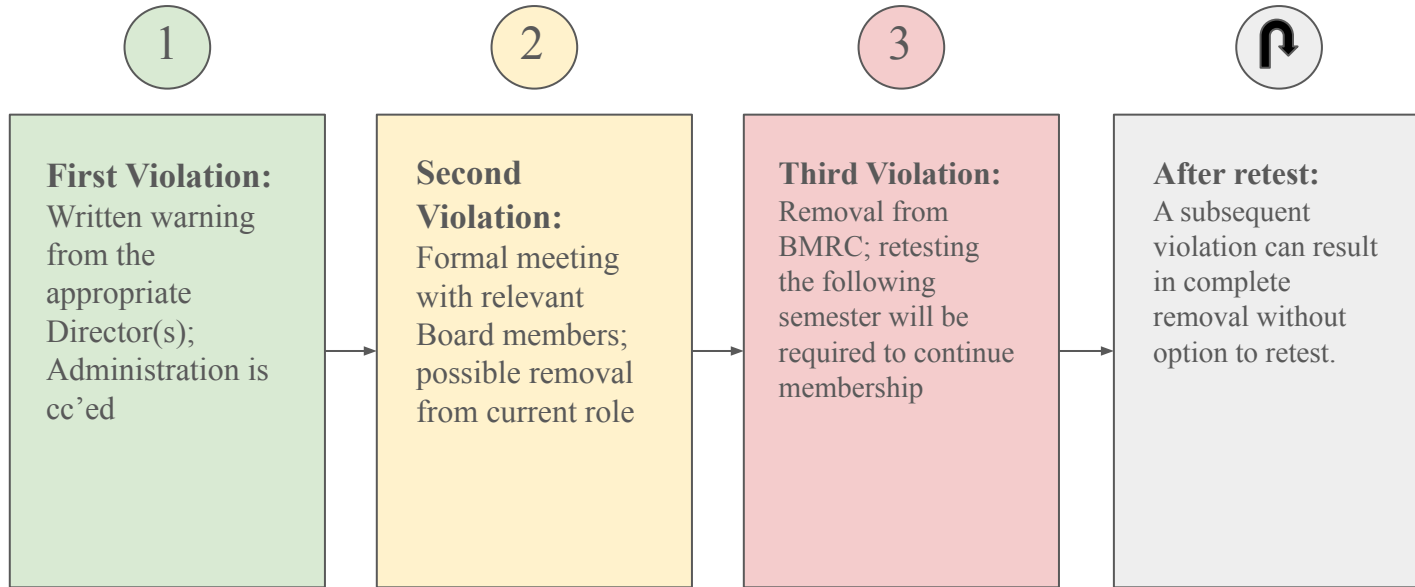


What counts as an event related violation?

Within first 15 minutes of call time	Minor violation; documented
30+ min after call time	Formal written warning
After scheduled class/event start	Formal written warning
No-show without explanation	May lead to probation/removal
Same-day cancellation without emergency	Violation; documented



Progressive Violation Process



Expectations

- Communicate early
 - We all understand that life comes up and things happen! If anything comes up don't hesitate to reach out. Last minute cancellations affect the entire team and communicating early gives us a chance to meet staffing needs
- Know your call time, event/class start time, and maintain your certifications
- Violations will be documented via email and in our master roster.

Questions? Please reach out to administration@berkeleymrc.org



Statpack Checkin and Checkout

- <https://bmrc-logistics.firebaseio.com/dashboard/>

